

How the campus fills

The business plan, which is mostly a plan for signing 45 consultants.

Dr Itunu Akinware, Group Chief Executive, Medbury Medical Services

Itunu,

The financial plan showed you that this campus can only be hurt in one place. You can overrun the building by 60 per cent and still make money. You can only miss the consultant target by a third. So this document is mostly about how 45 consultants get signed, and then how each clinical company wins its own patients.

The headline is that the number is smaller than it sounds. To sign 45 we have to reach **417 named consultants over 18 months**, which is about five a week. That is a job for one person with a budget, not a campaign.

What I would watch is not the total but the trial. A meeting converts nothing. A consultant who books one session at half rate and comes back within 30 days has signed, whatever the paperwork says. Everything here is built to get people into a room rather than into a conversation.

The four clinical companies each need a different route to market and I have set them out separately. Only one of them, longevity and infusion, can be sold before we open. That is why it matters more than its share of revenue suggests.

Debo Odulana

Founding Partner, Consult for Africa

01 / THE NUMBER

Worked backwards from 45, not forwards from hope.

Stage	Needed	Converts	What it means
Identified and contacted	417	40%	A named consultant, a warm route in, and a first approach made
Qualified conversation	167	45%	Wants sessional space, right specialty, can commit to a recurring slot
Books a trial session	75	60%	First session at half rate. This is the real conversion event, not the meeting
Signs, guest or tier	45	100%	Books a second time within 30 days. That is what counts as signed

417 named consultants over 18 months is about 23 a month, or 5 a week. That is the whole target. Every conversion rate above is an assumption and none of them is heroic: 40 per cent of named contacts give you a real conversation, 45 per cent of those want what we are selling, and 60 per cent of those who try a session come back. If the trial conversion is better than 60 per cent, and it should be once the rooms are finished, the whole funnel gets easier.

The signing profile is not flat

	Signed by then	In the quarter	Implied effort
Quarter 1, to month 5	3	+3	31 contacts in the quarter
Quarter 2, to month 8	12	+8	77 contacts in the quarter
Quarter 3, to month 11	22	+11	100 contacts in the quarter
Quarter 4, to month 14	33	+11	100 contacts in the quarter
Quarter 5, to month 17	42	+8	77 contacts in the quarter
Quarter 6, to month 20	45	+3	31 contacts in the quarter

The middle four quarters carry three quarters of the signings. Quarter one is slow because the rooms are new and there is nothing to show; the last quarter is slow because prime time is filling up. Judge the campaign on quarters two to five.

How big is the pool

	Penetration	Read
If Abuja holds 600 specialist consultants	45 is 7.5% of them	Needs a wide net
If Abuja holds 900 specialist consultants	45 is 5.0% of them	Needs a wide net

	Penetration	Read
If Abuja holds 1,200 specialist consultants	45 is 3.8% of them	Comfortable
The survey tells us what 20 consultants want. It does not size the pool, and that is the first gap named in the feasibility study. Sizing it is a six week piece of work and it should run alongside the fit-out, not after it.		

02 / WHO WE ARE SELLING TO

Four segments, and they do not want the same thing.

Segment	Size	What they actually want	How we reach them
Hospital consultants with privileges	About two thirds of the market	64% of the survey practise at a hospital where they have privileges and only 4% have their own rooms. They want an address and a nurse, not a lease	Direct approach through the medical directors and heads of unit. Specialty society meetings. Consultant to consultant referral once the first 10 are in
Specialists already in private practice	The fastest to convert	Already paying for space somewhere worse. The pitch is the address, the parking, the billing handled and no lease	Direct approach with a costed comparison against what they pay today
Sub specialists with no local theatre	The theatre segment	30% of the survey perform procedures needing a full theatre and 24% need sedation. They currently travel or turn work away	Theatre open days. A surgical anchor who brings his own list brings others
Diaspora and visiting consultants	Episodic, high value	Fly in for a week, need a fully serviced room and a booked list waiting. No interest in overheads at all	Professional bodies abroad, alumni networks, and the campus website carrying a visiting consultant page

03 / THE OFFER

Designed to get people into a room, not into a meeting.

	What it is	Why it is set that way
The trial	First session at half rate	No commitment, no membership, no conversation about tiers. The only ask is that they use a room once
The guest rate	NGN 35,000 an hour, rack	25% of the survey will not join anything. They are not a lost sale, they are a different product
Entry, NGN 350k	10% off the room	Where 38% of the survey sit. This is the volume tier and it should be the default recommendation
Practice, NGN 750k	20% off the room	For two or more sessions a week
Founding, NGN 1.5M	30% off the room, and priority prime slots	Only for the first 15. Prime time is the scarce asset and it is worth more than the discount
The referral	A month of free sessions	For any introduction that signs. Consultant to consultant is the best channel and it should be paid for

Prime time is the lever, not price. Demand concentrates in weekday evenings and Saturday mornings, and there are only 168 prime hours a week across the six rooms. Below about 50 consultants a guaranteed Tuesday and Thursday evening costs us nothing to give and is worth a great deal to someone with a hospital job. Past that it is the most valuable thing we own. Sell the founding tier on the slot, not on the discount, and sell it early while it is still free to give.

04 / CHANNELS AND MONEY

Where the NGN 39 M a year goes.

Channel	NGN M a year	What it buys
Consultant to consultant referral	12.0	Free, and the best channel we have. A signed consultant introducing one more is worth more than any campaign. Pay for it: a month of free sessions for a successful introduction
Open evenings at the campus	8.0	Monthly from month 2. They have to stand in the room. The survey says the address and how it feels is what 85% are buying
Specialty society and NMA meetings	6.0	Sponsor, present, and take the room list away. This is where the named list of 167 comes from
Direct approach through heads of unit	5.0	The BD officer's core job. Five contacts and two qualified conversations a week is the whole target
Digital: the site, the booking page, search	8.0	A consultant checking us at 11pm has to be able to see a room, a rate and a slot without calling anyone
Business development officer	9.0	In post from month 1, two months before the doors open. Two qualified conversations a week is the job
Campus demand generation	39.0	
Launch	NGN M	What it buys
Brand and identity	10.0	Name, marks, collateral, tone
Website and booking front end	8.0	A consultant must be able to see a room, a rate and a slot at 11pm without calling anyone
Photography, film and virtual tour	4.0	85% are buying the address and how it feels. They cannot feel a price list
Launch and consultant open evenings	8.0	Three events before opening, not one after
Launch marketing, one off	30.0	

05 / THE CLINICAL COMPANIES

Four businesses, four different routes to market.

Medlyfe Aesthetics and Regenerative NGN 285 M a year, 633 active patients a year at ~450k. Consumer, visual, repeat Instagram and TikTok led, before and after with consent, practitioner as the brand. Corporate wellness days for the professional women who are the core buyer. Cross referral from longevity and from the conversion clinic. Repeat is the whole model: injectables return every three to four months, so retention beats acquisition

Lyfe Place Hair Restoration NGN 220 M a year, 132 cases a year at ~1.67 M. High value, long consideration

6 to 12 months from first search to booking, so the funnel has to be patient. Testimonial and result led with real cases. Diaspora is a genuine segment and will travel. Private consultation, discreet entrance, and no waiting room overlap with the aesthetics clinic. A high acquisition cost is affordable here and nowhere else

Bariatric, Metabolic and Endoscopic NGN 263 M a year. Referral and demand led, and it steps up three times

Three products at three price points, and a patient can move up the ladder without leaving. **GLP-1 and dietetics** from month 3 at about 780k a year across 122 patients: demand is running ahead of supply and does not need creating, only capturing. **Swallowable balloons** from month 3 too, 24 cases at about 4.0 M, and this is the important one because it is a real bariatric procedure that needs no theatre, no anaesthetist and no endoscopy stack. **Endoscopic balloon, sleeve gastropasty and gastric botox** from month 9 when the stack lands, 12 cases at about 5.0 M plus minor work. Surgery only if the theatre is built.

Channel is search and pharmacy referral, endocrinology and primary care referral, and corporate health screening as the front door. Programme based and monthly, so the metric that matters is month four retention, not sign ups. The GLP-1 cohort is also the referral base for the balloons: a patient who plateaus on medication is already in the building and already trusts the team

Medlyfe Longevity and Infusion NGN 240 M a year, 267 members a year at ~900k. Corporate and pre-sellable

The only line that can be sold before the doors open, which is why it de-risks the ramp. Corporate wellness contracts and executive health packages, not consumer digital. Sell annual programmes to companies in Q1 and the suite opens with a book

Longevity and infusion is the one to start selling first. It is the only line that can be sold before we open, because it is programme based and corporate rather than walk in. Every annual programme sold in the fit-out period is revenue on day one and it is the cheapest de-risking available to the ramp. It is also the highest revenue per square metre on the campus.

The clinics fund their own patient acquisition from inside their margins, about NGN 81 to 101 M a year across the four, which is 8 to 10 per cent of their NGN 1008 M. The NGN 39 M of campus money above is for the building and the consultants, and the two should not be confused or spent twice.

06 / WHAT GOOD LOOKS LIKE

Five checkpoints, and what to do if one is missed.

When	What should be true	What it means if it is not
Month 1	BD officer in post. Named list of 60 built. Longevity corporate conversations opened	If the list is not built, nothing downstream happens
Month 3, opening	8 consultants signed. First open evening held. Longevity opens with a book of pre-sold programmes	Fewer than 5 signed means the proposition is wrong, not the effort
Month 6	18 signed. Trial to signed conversion measured for the first time	If trial conversion is under 40%, the rooms or the service are the problem
Month 12	33 signed. Referral producing at least a quarter of new signings	If referral is not working by now, the existing consultants are not happy
Month 20	45 signed. Prime time at about 90% and the founding tier closed	At this point the constraint is the building, and the ground floor break clause becomes the live question

One number to run this business on. Not revenue, not occupancy, not marketing spend: **consultants signed, reported monthly against this curve.** Everything else in the campus follows it. If it is on track the model holds and the capital comes home in month 33. If it is a quarter behind for two quarters running, that is the moment to change the offer rather than spend more, and the feasibility study shows what a slow ramp does to the return.

Debo Odulana Consult for Africa / +234 913 813 8553 / hello@consultforafrica.com

Conversion rates in section 01 are assumptions, not evidence, and are stated so they can be challenged. Demand evidence from the premium medipark consultant survey, n=20, 12 to 19 August 2026, against a target of 40 to 60; it does not size the consultant pool. Patient volumes in section 05 are derived from the revenue plan and average values, not from a comparable clinic's accounts. Marketing allocations are planning figures before any agency or media quotation.